

Your medication guide

PAIRS WITH YOUR RECOVERY INSTRUCTIONS

Pairs with your
procedure-specific
recovery guide

Your prescriber has chosen medications based on your procedure and health history. **Only take what you were actually prescribed.** This guide explains how and when to use each one. If you have any questions, call our office before taking anything.

Pain & swelling — the core schedule

Research shows that **ibuprofen (Advil, Motrin) combined with acetaminophen (Tylenol) works as well as prescription opioids** for most dental pain, with fewer side effects. Start before the numbness wears off. Take both together, on schedule, for the first 48–72 hours — don't wait for pain to come back.

EXAMPLE SCHEDULE · EVERY 6 HOURS · WITH FOOD

6 AM

Ibuprofen 600 mg
+ Tylenol 1000 mg

12 PM

Ibuprofen 600 mg
+ Tylenol 1000 mg

6 PM

Ibuprofen 600 mg
+ Tylenol 1000 mg

12 AM

Ibuprofen 600 mg
+ Tylenol 1000 mg

- Pick times that fit your day — just keep each dose 6 hours apart.
- **Max 24 hr:** Ibuprofen 3200 mg · Acetaminophen 4000 mg. Don't exceed either.
- Always take ibuprofen with food to protect your stomach.
- After 72 hours, step down to only as needed.

If you were also prescribed...

JOURNAVX (SUZETRIGINE)

Non-opioid prescription pain medication

A newer non-opioid option for moderate to severe pain.

Non-addictive, no drowsiness, no respiratory risk. Take exactly as prescribed:

First dose: 100 mg on an empty stomach (at least 1 hour before or 2 hours after food). Clear liquids are okay.

After that: 50 mg every 12 hours, with or without food. Swallow whole — don't crush or chew.

Avoid grapefruit while taking this. Tell us if you start any new antibiotic, antifungal, or HIV medication — some interact.

OPIOID PAIN MEDICATION

Hydrocodone (Norco) or oxycodone (Percocet)

Only take this if the ibuprofen + Tylenol combination isn't enough. It's a backup, not your first line. Take 1 tablet every 6 hours as needed.

Don't combine with alcohol, sleeping pills, or other opioids. Don't drive. Most patients need this for 1–2 days at most, if at all.

Take leftover tablets to a pharmacy drug take-back bin — don't keep them in the house.

ANTIBIOTIC

Amoxicillin, Augmentin, or clindamycin

Start 2 days before your surgery and take the full course, even if you feel fine — don't stop early. Set reminders so you don't miss a dose.

Space doses evenly (for example, 8 AM / 2 PM / 8 PM / 2 AM for 4×/day, or 8 AM and 8 PM for 2×/day).

Mild stomach upset or diarrhea is common. Severe diarrhea, rash, or trouble breathing — stop the medication and call us.

STEROID (MEDROL DOSE PACK)

Methylprednisolone, tapered over 6 days

Follow the printed instructions on the package exactly — the dose goes down each day. **Take all doses with food.**

While you're on the steroid pack, stop taking ibuprofen. Use acetaminophen (Tylenol) alone for pain. Resume ibuprofen the day after the pack is finished.

You may feel wired, have trouble sleeping, or notice an odd taste. All normal and temporary.

DECONGESTANT

Pseudoephedrine (Sudafed) — for sinus procedures

Take as directed for 5–7 days. Keeps your sinus draining properly while the area heals.

Don't use Afrin or similar nasal sprays for more than 3 days — they cause rebound congestion. Saline nasal spray is safe to use as often as you want.

Skip if you have high blood pressure, heart issues, or prostate problems — call us for an alternative.

CHLORHEXIDINE RINSE

Peridex, Paradex, or Periogard (prescription)

Rinse **twice daily** — morning and night — **starting 2 days before your surgery**, and continue after surgery for the time on your recovery guide (usually 2 weeks) — skip the dose on the evening of surgery (no rinsing for 24 hours) and restart the next morning. Unlike your antibiotic, you don't need to finish the bottle. Don't swish forcefully; let it pool, tilt your head gently, then let it fall out. **Bone graft or implant:** use it as a 1-minute soak held over the site instead of a rinse for 1 week after surgery, then a saturated cotton swab wiped gently along the stitch line for 2 more weeks — see your recovery guide.

Don't eat or drink for 30 minutes after. Use salt water the rest of the day.

Can stain teeth brown temporarily. A cleaning after you're done removes it.

ANTI-NAUSEA MEDICATION

Ondansetron (Zofran) — if prescribed

Take only if you feel nauseated. Dissolves under the tongue — no water needed. Can take every 8 hours as needed. Not everyone needs this; if you feel fine, you don't need to take it.

Optional supplements that may help

PROBIOTIC

If you're taking an antibiotic

Antibiotics cause diarrhea in about 1 in 5 people. A probiotic cuts that risk roughly in half.

Choose a specific strain: *Saccharomyces boulardii* (Florastor) or *Lactobacillus rhamnosus* GG (Culturelle). Once daily, starting the same day as your antibiotic, continuing 3–5 days after it's done.

Space 2 hours apart from your antibiotic dose. Skip if immunocompromised, on chemotherapy, or if you have a central IV line.

BROMELAIN

Optional, for swelling

A pineapple-derived enzyme with research support for reducing swelling after oral surgery. **500 mg, 3× daily, for 5–7 days.** Look for products with at least 2,000 GDU/g.

Skip if you take a blood thinner, have a bleeding disorder, or are allergic to pineapple.

Important: Don't mix prescription medications with alcohol, recreational drugs, or over-the-counter sleep aids without checking with us. If you're not sure whether something is safe to take, call first.

Acknowledgment & signature

I acknowledge that I have received and understand these instructions, and all my questions have been answered. I agree to follow them and to call the office with any questions or concerns.

PATIENT SIGNATURE

DATE

PATIENT PRINTED NAME

DATE OF BIRTH

PARENT / LEGAL GUARDIAN / CAREGIVER SIGNATURE (IF SIGNING FOR THE PATIENT)

DATE

PRINTED NAME

RELATIONSHIP TO PATIENT